

FLUCOR NIGHT PE SOFT GELATIN CAPSULE

WIFLU07-var (MY)

DESCRIPTION

Oblong-shaped, green transparent soft gelatin capsule printed with "Flucor PE", filled with transparent green liquid.

COMPOSITION

Each soft gelatin capsule contains:

Paracetamol	250.0 mg
Phenylephrine HCl	5.0 mg
Chlorpheniramine maleate	2.0 mg

ACTIONS AND PHARMACOLOGY

Paracetamol, as an analgesic, may act predominantly by inhibiting prostaglandin synthesis in the central nervous system (CNS) and, to a lesser extent, through a peripheral action by blocking pain-impulse generation. The peripheral action may also be due to inhibition of prostaglandin synthesis or to inhibition of the synthesis or actions of other substances that sensitize pain receptors to mechanical or chemical stimulation. Paracetamol probably produces antipyresis by acting centrally on the hypothalamic heat-regulating center to produce peripheral vasodilation resulting in increased blood flow through the skin, sweating, and heat loss. The central action probably involves inhibition of prostaglandin synthesis in the hypothalamus.

Phenylephrine hydrochloride is a sympathomimetic with mainly direct effects on adrenergic receptors. It has predominantly alpha-adrenergic activity and is without significant stimulating effects on the CNS at usual dose. Following oral administration or topical application of phenylephrine to the mucosa, constriction of blood vessels in the nasal mucosa relieves nasal congestion associated with allergy or head colds.

Chlorpheniramine maleate is a sedating antihistamine that causes a moderate degree of sedation. Its anticholinergic actions provide a drying effect on the nasal mucosa.

PHARMACOKINETICS

Paracetamol is readily absorbed from the gastrointestinal tract with peak plasma concentrations occurring about 10 to 60 minutes after oral administration and is distributed into most body tissues. The elimination half-life of paracetamol varies from about 1 to 3 hours. It is metabolized predominantly in the liver and excreted in the urine mainly as the glucuronide and sulphate conjugates, less than 5% is excreted as unchanged paracetamol.

Phenylephrine has a low bioavailability owing to irregular absorption and first-pass metabolism by monoamine oxidase in the gut and liver.

Chlorpheniramine maleate is absorbed relatively slowly from the gastrointestinal tract, peak plasma concentrations occurring about 2.5 to 6 hours after oral administration. It appears to undergo considerable first-pass metabolism. About 70% of chlorpheniramine in the circulation is bound to plasma proteins. Values ranging from 2 to 43 hours have been reported for the half-life. Chlorpheniramine maleate is widely distributed in the body and is extensively metabolized. Metabolites include desmethyl- and didesmethyl-chlorpheniramine. Unchanged drug and metabolites are excreted primarily in the urine; trace amounts found in the faeces.

INDICATIONS

For the temporary relief of running nose and sneezing, minor aches and pains, headache, fever and nasal congestion associated with the common cold.

CONTRAINDICATIONS

- In patients with a known hypersensitivity to Paracetamol, Phenylephrine or Chlorpheniramine.

- In patients under treatment with MAO inhibitor and within 2 weeks of stopping such treatment.
- In patients with severe hypertension or severe coronary heart disease.

WARNING

When used for treatment of Cough and Cold:

- Not to be used in children less than 2 years of age.
- To be used with caution and doctor's/ pharmacist's advice in children 2 to 6 years of age.

This preparation contains PARACETAMOL. Do not take any other paracetamol containing medicines at the same time.

Do not take Flucor Night PE if you have taken a monoamine oxidase inhibitor (MAOI) such as isocarboxazid, phenelzine, tranlycypromine or selegiline in the last 14 days.

This medicine may cause drowsiness. Caution when driving or operating machinery.

Allergy alert: Paracetamol may cause severe skin reactions. Symptoms may include skin reddening, blisters or rash. These could be signs of a serious condition. If these reactions occur, stop use and seek medical assistance right away.

PRECAUTIONS

- Paracetamol should be given with care to patients with impaired kidney or liver function and to patients with alcohol dependence.
- Phenylephrine should be avoided in severe hyperthyroidism and used with caution in severe ischaemic heart disease.
- Chlorpheniramine maleate will cause drowsiness. Those affected should not drive or operate machinery; alcohol should be avoided. It should be used with care in conditions such as glaucoma, urinary retention and prostatic hyperplasia.
- Do not exceed recommended dosage. Consult general practitioner should symptoms persist.
- Patients with heart disease, thyroid disease, high blood pressure, breathing problems, persistent or chronic cough, cough with excessive phlegm, diabetes, glaucoma or difficulty in urination due to enlargement of the prostate gland should consult a doctor before using this product.

MAIN SIDE/ADVERSE EFFECTS

- Cutaneous hypersensitivity reactions including skin rashes, angioedema, Stevens Johnson Syndrome/Toxic Epidermal Necrolysis have been reported.
- Haematological reactions including thrombocytopenia, leukopenia, pancytopenia, neutropenia and agranulocytosis have been reported as side effects of paracetamol with skin rashes and other hypersensitivity reactions occur occasionally.
- The commonest adverse effects of phenylephrine include anxiety, palpitations, arrhythmias, headache, cold extremities/gangrene, hypertension, nausea, vomiting, sweating, weakness, fear, restlessness, insomnia, confusion, irritability, psychotic states, dyspnoea, anorexia, extravasation causing tissue necrosis and sloughing, mydriasis, difficulty in micturition and urinary retention, piloerection, increased salivation, hyperglycaemia and lactic acidosis.
- Adverse effects with chlorpheniramine appear to be less frequent or rare and may include difficult or painful urination, drowsiness, increased sweating, loss of appetite and paradoxical reaction.

PREGNANCY AND LACTATION

Safety and efficacy for use during pregnancy and lactation has not been established.

DRUG INTERACTIONS

Concurrent use of Flucor Night PE and the following drugs must be avoided or if necessary, use with care:

- Alcohol
- Antidepressants
- Antihypertensive drugs
- Amphetamine
- Appetite suppressants
- Hepatic enzyme inducers
- Hepatotoxic medications
- MAO inhibitor

OVERDOSE

Symptoms:

Toxic symptoms include vomiting, abdominal pain, hypotension and sweating. The most serious adverse effect of acute overdose of Paracetamol is a dose-dependent, potentially fatal hepatic necrosis. In adults, hepatotoxicity may occur after ingestion of a single dose of 10 to 15g of Paracetamol, a dose of 25g or more is potentially fatal.

Symptoms during the first 2 days of acute poisoning by Paracetamol do not reflect the potential seriousness of the intoxication. Major manifestations of liver failure such as jaundice, hypoglycemia and metabolic acidosis may take at least 3 days to develop.

TREATMENT FOR OVERDOSE

In cases of overdose, methods of reducing the absorption of ingested drug are important. Gastric lavage is essential even if several hours have elapsed. Prompt administration of 50g activated charcoal and 500mL iced mannitol 20% by mouth may reduce absorption. If the history suggests that 15g Paracetamol or more has been ingested, administer one of the following antidotes:

- Acetylcysteine 20% IV
- Oral methionine
- Oral acetylcysteine 5%

DOSAGE AND ADMINISTRATION

Oral.

Adults and children 12 years of age and over:

2 softgels every 4 hours; not exceeding 8 softgels in 24 hours.

Note:

The information given here is limited. For further information, consult your doctor or pharmacist.

Storage:

Store below 30-C.

Protect from moisture.

Presentation/Packing:

Blister pack of 1 x 8's.

Product Registration Holder /

Manufactured by: HOVID Bhd.

121, Jalan Tunku Abdul Rahman, 30010 Ipoh, Malaysia.

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